



OFFICIAL

NMHS MHS PROCEDURE

Safe Use and Storage of Ligature Cutters

Scope (Audience)	All NMHS Mental Health Services (MHS) Staff
Scope (Area)	All NMHS MHS Inpatient Units
Summary	To provide direction regarding the safe use, storage and monitoring of ligature cutters in clinical areas.
Risk Rating	Medium
NSQHS Standards	Clinical Governance (Std 1) Communicating for Safety (Std 6)
Policy Sponsor	Director of Operations, Mental Health
Policy Contact	Director of Operations, Mental Health
Version	3.0
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COMPLIANCE WITH THIS DOCUMENT IS MANDATORY

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1. Aim

To provide direction regarding the safe use, storage and monitoring of ligature cutters in clinical areas.

2. Background

The National Safety and Quality Health Service Standards (NSQHSS) the [MP 0006/16 WA Department of Health Risk Management Policy](#) and the [Department of Health Clinical Risk Management Guidelines – a best practice guide](#), identify the requirement for health services to provide safe environments for patients, staff and visitors for the delivery of health care.

There are many complexities to ensuring a safe environment for patients of health services. This is particularly challenging within mental health inpatient settings due to the design and layout of inpatient environments and the nature of associated clinical risks such as self-harm, suicide attempts and completed suicides as identified in:

- [MP 0181/24 Safety Planning for Mental Health Consumers Policy](#) provides a framework for collaboratively developing safety plans with patients at risk of self-harm or suicide, ensuring care is individualised and supports autonomy.
- [MP 0155/21 Statewide Standardised Clinical Documentation for Mental Health Services](#), which mandates the use of Risk Assessment and Management Plan (**RAMP**) for documenting and managing clinical risks across WA Mental Health Services, ensuring consistency and accountability.
- [MHS Clinical Risk Assessment and Management Policy](#).
- [Principles and Best Practice for the Care of People Who May Be Suicidal](#), emphasises evidence-based suicide prevention strategies, risk formulation, and recovery-oriented care.
- [Guidelines for People Who May Be at Risk of Exhibiting Violent or Aggressive Behaviour](#).

3. Definitions

Ligature	Something which binds or ties, specifically, something which could potentially be used or has been used for self-strangulation.
Ligature Point	Any fixture, fitting or environmental feature capable of supporting body weight or creating sufficient tension to cause harm through hanging or strangulation.
Partial Suspension	A situation where only part of the body weight is supported by the ligature.
Full Suspension	A situation where the full body weight is supported by the ligature.

4. Risk

Non-compliance with this procedure will impact on the following risk categories:

Culture	☐	Organisational Development	☐
Emergency Management	☒	Policy & Legislative Compliance	☒
Fiscal Responsibility	☐	Quality of Patient Care	☒
Governance & Accountability	☐	Research & Innovation	☐

Information Communications Technology	☐	Staff Safety & Wellness	☒
Facility Management	☐	Workforce Planning	☐
Medical Equipment	☐	Other	☐

5. Key Points

- Ligature cutters are essential emergency equipment and are to be stored in a place that is identified and accessible to staff.
- All clinical staff will receive education regarding ligature cutters as part of local orientation to the clinical area.
- Annual mandatory training must include practical demonstration and simulated use of ligature cutters where possible (for example during Code Blue training or during INMART).
- Agency, casual or redeployed staff must receive local orientation regarding the location and use of ligature cutters prior to commencing clinical duties within the clinical area.
- Ligature cutters must never be stored in patient accessible areas.

6. Daily Equipment Checks

- The Shift Coordinator (or delegated Registered Nurse) is responsible for confirming the presence and functionality of ligature cutters at the commencement of each shift.
- The check must be documented on the Ward Daily Checklist in accordance with local clinical governance requirements.
- If any ligature cutter is missing, damaged or not functional, this must be escalated immediately to the Nurse Manager or After Hours Clinical Nurse Manager Ward Report.
- Each site carries a spare set of ligature cutters accessible by the After-Hours Clinical Nurse Manager to be used until permanent replacement can be arranged.
- Replacement equipment must be arranged immediately.
- To request replacement of a piece of equipment email Emergency and Operations team (mhemops@health.wa.gov.au) with type of cutters requiring replacement and ward cost centre.

7. Equipment

- All areas will have three (3) types of ligature cutters available to ensure all potential materials can be removed:
 - Res-Q-hook
 - Wire Cutter
 - Trauma Scissors
- Ligature cutter kits must be stored in a locked staff-only access point (for example within the nursing station) and must not be visible or accessible to consumers or visitors.
- All ligature cutters should only be used for ligature removal and not for any other purpose to prevent the risk of them going blunt. All ligature cutters contain sharp components and must be handled with care; staff should avoid direct contact with blades and use caution during both operation and cleaning.

Res-Q-hook (or similar) which is the main tool which is specifically a ligature cutter. For most ligatures a cutting action, rather than a pulling action, is required to effectively break the ligature.

Img 1. Res-Q-Hook



Wire Cutter for a situation in which someone may use a metal cord (e.g. TV Antenna cable)

Img 2. Wire Cutter



Trauma scissors (also known as medical emergency shears) are a type of scissors used by emergency workers to quickly and safely cut clothing from injured people. The serrated blade is ideal for cutting through clothing, bandages or other materials and the blunt edge of the shears keeps patients safe from accidental cuts or other injuries.



8. Procedure

8.1 Responding to a Person with a Ligature in Situ

1. Immediately call for assistance and activate your personal duress alarm (PDA) or duress device. Ask another staff member to activate a Code Black – Self Harm.
2. Assess whether it is safe to approach the person or wait for assistance.
3. Check the area for hazards (i.e. weapons, glass, water, electricity) and ensure that the area is safe before approaching the person.
Staff must undertake a dynamic risk assessment prior to intervening.
4. Where possible, assistance should be sought immediately and coordinated team intervention undertaken. Lift the patient or support the person's body weight (if safe to do so) as soon as possible, to release the tension on the ligature.
5. All ligature suspension incidents must be treated as high risk manual handling events. Staff should not place themselves at unnecessary physical risk while attempting to support body weight or remove the ligature.
6. Direct staff attending the scene to obtain the ligature cutters and activate a Code Blue as per the Emergency Procedures Manuals.
7. If the person has used a power cord as a ligature do not cut through a power cord until it is unplugged from the socket.
8. Use the ligature cutters to remove the ligature and lower the patient to the floor in a controlled manner.
9. If the ligature cannot be removed keep the body weight supported to take tension off the ligature until the ligature has been removed.
10. If the person resists staff attempts to remove the ligature, it may be necessary for staff to use appropriate physical intervention (restraint) to ensure the safe removal of the ligature.
11. Administer CPR if required at the earliest opportunity.
12. In the event of a Code Blue activation refer to the [Emergency response for Code Blue management](#).
13. The Medical Officer responding will lead all medical interventions including airway management and direct ongoing care.
14. Lack of visible injury following non-fatal strangulation should not influence decision making regarding medical review and radiological investigation.
15. Complete the Medical Emergency (ME) Code Blue Form on the [Code Blue reporting data base](#).
16. Record the incident in the Clinical Incident Management System (Datix-CIMS) and in the progress notes in the Patient's Medical Record.
17. Reporting of the incident will be in accordance with the [NMHS Clinical Incident Management Policy and site](#), MHS Inpatient Emergency Management procedures and Local Area Action Plans.

8.2 Post Incident Management

- Following an incident the consumer involved must be reviewed by a medical officer for physical and mental state deterioration and an appropriate plan documented in the medical

record. The Treatment Support and Discharge Plan and Risk Assessment and Management Plan should be updated following all incidents involving a ligature.

- Arrange a replacement ligature cutter for the area and return the ligature cutters to the identified storage area in the ward.
- Following use, ligature cutters must be handled using appropriate Personal Protective Equipment (PPE).
- If required for Police or Coroner investigation, the equipment must be secured in a safe location and must not be returned to circulation which may include Coordinator of Nursing office, Service Manager office in a bag or box.
- If the equipment is not required for investigation, ligature cutters must be decontaminated in accordance with [MHS IPC Environmental Cleaning Policy](#).
- Prior to returning the equipment to storage, cutters must be inspected to ensure the blades remain intact, sharp and fully functional.
- Any cutter identified as blunt, damaged or compromised must be removed from circulation and replaced immediately.

9. Defusing and Debriefing

9.1 Defusing/Debriefing for Patients

Reassure and support patients in the vicinity of the incident or patients who are aware of the event.

Ensure that allocated nurses are aware of the event and are vigilant in undertaking assessment and monitoring of any changes in mental state or behaviours following the incident and are responsive and supportive towards their allocated patients.

9.2 Hot Debrief and Operational Debrief

At the conclusion of an incident, once all staff have returned to baseline, it is the responsibility of senior staff present to ensure that a hot debrief has been offered. The goal of a hot debrief is to reflect on operational processes and decisions involved in a non-judgemental environment. The facilitator must maintain psychological safety and ensure a no-blame approach is followed. This is also an opportunity to identify if there are further issues that require operational debriefing or psychological first aid.

9.3 Psychological First Aid

Following an incident, managers must ensure immediate check in occurs to ensure all staff are safe to continue duties or sent home with appropriate support if required. Managers should utilise the principles of psychological first aid as per the [NMHS Critical Incident Response SharePoint Page](#) to provide wellbeing support for staff as appropriate. The goal of psychological first aid is to promote natural resilience and recovery of individuals and to prevent acute distress reactions developing into long term distress. Support available includes but is not limited to:

- Voluntary psychological first aid session facilitated by wellbeing psychologists
- 1:1 support through Wellbeing Psychologists
- Access to the Employee Assistance Program (EAP)
- Referral to the NMHS HSW Injury Management Department

10. Monitoring and Evaluation

Compliance with this procedure is mandatory.

Implementation and compliance with this procedure will be monitored by the Mental Health Emergency and Operations Committee, which meets monthly to review performance, incidents and any required improvement actions

This procedure will be evaluated as required to determine its currency, relevance, effectiveness, efficiency, and sustainability. At a minimum, it will be reviewed annually using a risk management approach in accordance with the [NMHS Policy Governance Framework](#).

11. Related Documents

WA Health policies	• MP 0122/19 – Clinical Incident Management Policy • Clinical Risk Management Guidelines • Principles and Best Practice for the Care of People Who May Be Suicidal • Clinical Risk Management Guidelines • MP 0181/24 Safety Planning for Mental Health Consumers Policy
NMHS Policy documents	• NMHS Clinical Incident Management Policy
NMHS MHS Policy documents	• MHS Clinical Risk Assessment and Management Policy • MHSDHS Mandatory Training Policy • MHS IPC Environmental Cleaning Policy

12. Document Control

Document Version Control			
Date	Version	Author	Notes
16/11/2018	1.0	Nurse Director SFMHS	Initial Endorsement
11/12/2018	1.1	Director of Nursing	Sponsor and SQ&P Manager approved – Minor amendment: change of policy type, reformatting of document, added references. No change to content
18/05/2022	2.0	Director of Nursing	Scheduled review, procedure updated to standardise equipment, practices, orientation and education across all inpatient MH services.
18/03/2026	3.0	Director of Operations MHS	Scheduled review, Sponsorship changed to Director of Operations/Emergency Management, equipment management, checking process, supply and staff debriefing process added.

This document can be made available in alternative formats on request for a person with a disability.

Within the Department of Health WA, the term Aboriginal is used in preference to Aboriginal and Torres Strait Islander, in recognition that Aboriginal people are the original inhabitants of Western Australia. No disrespect is intended to our Torres Strait Islander colleagues and community.

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